- Nervous system
- Kidneys.
- Neurological (Encephalitis encephalopathy)
- Prolonged convulsions
- Infantile spasm

Control measures

1. Cases and carriers

a) Early detection: Carriers can be detected only by culture method. Swabs can be taken from both the nose and throat and examined by culture methods for diphtheria bacilli.

b) Isolation: Suspected cases and carriers should be promptly isolated, preferably in a hospital for at least 14 days.

c) Treatment: For Cases when diphtheria is suspected diphtheria antitoxin should be given without delay. IM or IV in doses ranging from 20,000 to 1,00,000 units or more depending upon the severity of the case.

2. For Carriers : The carriers should be treated with 10 days course of oral erythromycin which is the most effective drug for the treatment carriers.

a. Contacts : Contacts need special attention. They should be throat swabbed and their immunity status determined. The bacteriological surveillance of close contact should be continued for several weeks.

b. Community : The only effective control is by active immunization with diphtheria toxoid. All children who are not previously immunized should be given a dose of 500 to 1000 IV diphtheria antitoxin.

Treatment

- The specific treatment is diphtheria antitoxin which must be given immediately in doses ranging from 10,000 to 80,000 units according to the severity of the case.
- Antibiotics (penicillin) help to eliminate the infection and prevent production of further toxin.
- Bed rest is essential to prevent heart failure.
- Tracheostomy may be needed if there is respiratory obstruction.

Preventive measures

- Diphtheria can be prevented by active immunization either by DPT or diphtheria vaccine.
- The current practice is to immunize all infants with DPT starting from the age of 6weeks.
- DPT vaccine protects not only against diphtheria but also against pertussis and tetanus.
- A booster dose (0.5 ml) of DPT is recommended at the age of 1½ to 2 years followed by another dose (DT only) at the age of 5 and 6 years.

Whooping Cough

Definition

A highly infectious disease of the respiratory tract, caused by the whooping cough bacilli. The disease occurs in epidemics every 3-4 years.

Causative organism

Whooping cough bacilli Bordetella pertussis.

Mode of transmission - Droplet method and by personal contact.

Incubation period - Usually 7 to 14 days but not more than 3 weeks

Clinical manifestations

a. Catarrhal Stage
b. Paroxysmal Stage
c. Convalescent Stage.

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